

The Caregiving Gap

Singapore isn't short on respite care services. It's short on caregivers who know those services exist, or who can find the one that fits their week and their wallet. That gap — not a shortage of supply — is what Respite Navigator is built against.

20.7% → 23.9% of citizens aged 65+, 2025 → 2030

74,000 → 152,000 people living with dementia, 2023 → 2030

83% of caregivers aware of respite care have never used it

§1 The demographic wave — why this can't wait

Two trends moving in the same direction, both accelerating.

20.7% → 23.9%

Singapore citizens aged 65 and above — up from 13.1% just a decade earlier (June 2015), and forecast to reach roughly **1 in 4 citizens** by June 2030.

Singapore Department of Statistics, via population.gov.sg

74,000 → 152,000

Persons living with dementia in Singapore, 2023 vs. the Ministry of Health's own 2030 projection — a **105% increase** in seven years, each case typically supported by one or more informal caregivers.

[Ministry of Health](#), citing the IMH WiSE Study

§2 What caregiving already costs, before any solution

This is the burden Respite Navigator is trying to lighten — measured, not assumed.

210,000+

Estimated caregivers in Singapore today — roughly the population of Bishan and Toa Payoh combined.

AWARE, 2019 report — cited in ["The Big Read," Malay Mail/CNA, 2021](#)

3 in 4

Caregivers of a person with mental health issues report feeling tired from caregiving.

SMU 2020 survey, cited by [Ministry of Health](#)

S\$56,877 / yr

Average annual income loss for caregivers whose work arrangements changed because of caregiving — S\$582,572 on average over four years. 63% of those who changed their work reported an income loss.

AWARE, 2019 — via [Malay Mail/CNA "The Big Read," 2021](#)

33.99%

Of caregivers looking after an older adult report two or more chronic health conditions of their own — the physical cost of the role, showing up in the caregiver's own body.

SMU ROSA Research Brief, [Mar 2025](#)

§3 The real bottleneck: awareness, not availability

This is the thesis the whole product rests on — and it holds up even in a country that already solved the funding and directory problem.

50.09%

Of informal caregivers in Singapore are even aware that respite care services exist for them to use.

SMU ROSA Research Brief, [Mar 2025](#)

82.83%

Of the caregivers who *are* aware respite care exists have still never used it — awareness alone doesn't close the gap.

SMU ROSA Research Brief, [Mar 2025](#)

WHY THIS ISN'T JUST A SINGAPORE PROBLEM TO SOLVE FROM SCRATCH

Australia already built the "obvious" fix — a national, government-funded carer-support and respite-referral service called **Carer Gateway** — and its own impact evaluation found the identical gap.

INTERNATIONAL CHECK · AUSTRALIA'S CARER GATEWAY

A mature, fully funded, nationally advertised digital carer-support service — the closest real-world precedent to Respite Navigator's endpoint — and even it struggles to close the awareness-to-usage gap.

6% of Australian carers had accessed Carer Gateway by June 2023, despite national funding and years of operation

largest gain in carer wellbeing came specifically from respite access — the single most impactful service type, when carers actually reached it

70.9% of carers who used its digital counselling recorded a positive change

Australian Government Department of Health, Disability and Ageing — [Impact Evaluation of the Integrated Carer Support Service](#) (2025)

The takeaway for the pitch: respite care *works* — it produces the biggest measurable wellbeing gain of any carer support type — but building the directory was never the hard part. Getting the right caregiver to the right service, at the moment they need it, is. That's the gap a matching agent is built to close, not a bigger directory.

§4 Real voices behind the numbers

Five Singapore caregivers who spoke publicly to CNA/TODAY about their experience, as reported in ["The Big Read: As Singapore society ages, who will care for the caregivers?"](#) (2021). Each is tagged with the part of Respite Navigator's design it speaks to most directly.

BS

Belinda Seet, 63

Part-time lecturer, primary caregiver for her 89-year-old mother with Alzheimer's

Says she cannot afford respite care priced at S\$20–S\$40 an hour — the cost alone rules it out before she can even assess whether a service would fit her mother's needs.

→ budget-first matching, not just proximity

JC

Jasmine Chua, 49

Caregiver for her 84-year-old mother, who has dementia

Lost sleep to her mother's nighttime waking and eventually quit her part-time clinic job to provide full-time care — a decision made without ever weighing a respite option against it.

→ surfacing options before the caregiver quits work

Reported in Malay Mail/CNA, "The Big Read," 2021

HK

Henry Koh, 43

Caregiver for his 76-year-old mother and 95-year-old grandmother, both living with dementia

Left his job as a behavioural therapist and took a significant pay cut to manage caregiving for two family members at once — exactly the multi-recipient, multi-schedule case a matching agent needs to handle, not simplify away.

→ profiles for more than one care recipient

Reported in Malay Mail/CNA, "The Big Read," 2021

DT

Danny Raven Tan, 53

Caregiver for his 88-year-old mother, diagnosed with dementia in 2015

Describes going in "not knowing anything about dementia," a knowledge gap that eventually led him to found a dementia-awareness organisation himself — the exact plain-language navigation gap the product's intake and matching steps are meant to close for someone earlier in that journey.

→ plain-language intake, no clinical jargon required

Reported in Malay Mail/CNA, "The Big Read," 2021

MF

Madam Fadilah, 57

Homemaker, caregiver for her 86-year-old mother-in-law with Alzheimer's

Has spoken publicly about reaching a breaking point during a pandemic lockdown, when caregiving strain went unaddressed for too long. Cited here only to make one point concrete: unmet caregiver strain is not a hypothetical harm — it is the specific harm early access to respite is meant to prevent.

→ the cost of the status quo, stated plainly

§5 What this means for the build

Every number above maps to a specific decision already in the Respite Navigator workflow — this is the paragraph to lift straight into the deck's "why it matters" slide.

83% never used → the product's whole premise is closing an **awareness-to-usage** gap, not building a bigger directory of services that already exist.

\$20–40/hr → budget is a first-class filter in the intake step, not an afterthought — Belinda Seet's story shows cost alone can end the search before it starts.

63% income loss → the product has to work in the five minutes a caregiver actually has, not require a form that assumes free daytime hours.

6% Carer Gateway uptake → even a fully funded national precedent proves supply isn't the bottleneck — validates building the matching layer, not another directory.

§6 Sources

01 Singapore Department of Statistics population trends — population.gov.sg, "[Longevity](#)"

02 Ministry of Health — [dementia projections to 2030](#)

03 Ministry of Health — [data on physical and mental health of caregivers](#)

04 SMU ROSA Research Brief, "[Profiles and Well-Being of Caregivers in Singapore](#)," March 2025 (PDF).

05 Malay Mail / CNA — "[The Big Read: As Singapore society ages, who will care for the caregivers?](#)", 2021

06 Australian Government Department of Health, Disability and Ageing — [Impact Evaluation of the Integrated Carer Support Service \(Carer Gateway\)](#), 2025

been updated since this brief was compiled.